

IMMIGRANT MENTAL HEALTH DATA SHEETS

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Source Mental Health and Access to Care Survey (MHACS):
Public Use Microdata File. DOI:
<https://doi.org/10.25318/82M0021X-eng>

INTRODUCTION

Immigrating to a new country is a transformative experience that brings both opportunities and challenges. Mental health plays a crucial role in how individuals adapt to life in Canada, yet many newcomers may face stress, isolation, or uncertainty during their transition. This poster uses data from Statistics Canada to provide clear, accessible insights into the mental health experiences of immigrants in Canada. By presenting this information in an easy-to-understand format, we aim to empower newcomers with knowledge, reduce stigma, and encourage engagement with the community.

METHODOLOGY

Data Source: The data presented in this poster is drawn from Statistics Canada surveys on mental health, such as the Mental Health and Access to Care Survey 2022 Public Use Microdata File (PUMF).

Simplification: Complex data points were translated into simplified, visually engaging charts, infographics, and key takeaways to ensure accessibility for diverse audiences.

Design: The poster was designed with clear language and accessible visuals.

Focus Areas: The poster highlights statistics on common mental health challenges. We focused on data from immigrants and explored indicators such as stress, access to mental health resources, and community participation.

OBJECTIVE

To transform complex government data about immigrant mental health in Canada into accessible, engaging visualizations.

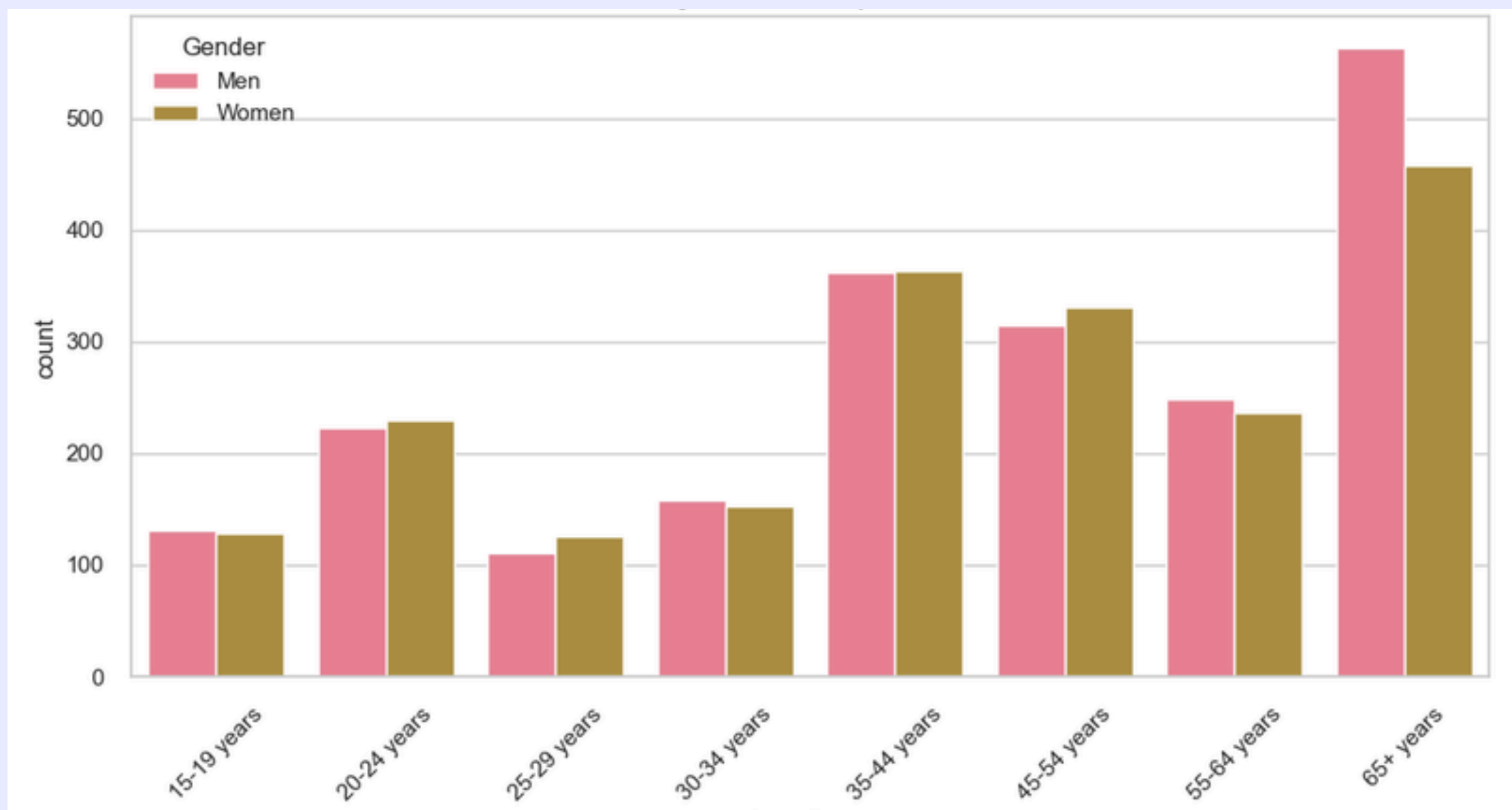
To present objective, evidence-based information about immigrant mental health in Canada in a clear and visually engaging way.

To equip newcomers with knowledge to better understand and address mental health challenges.

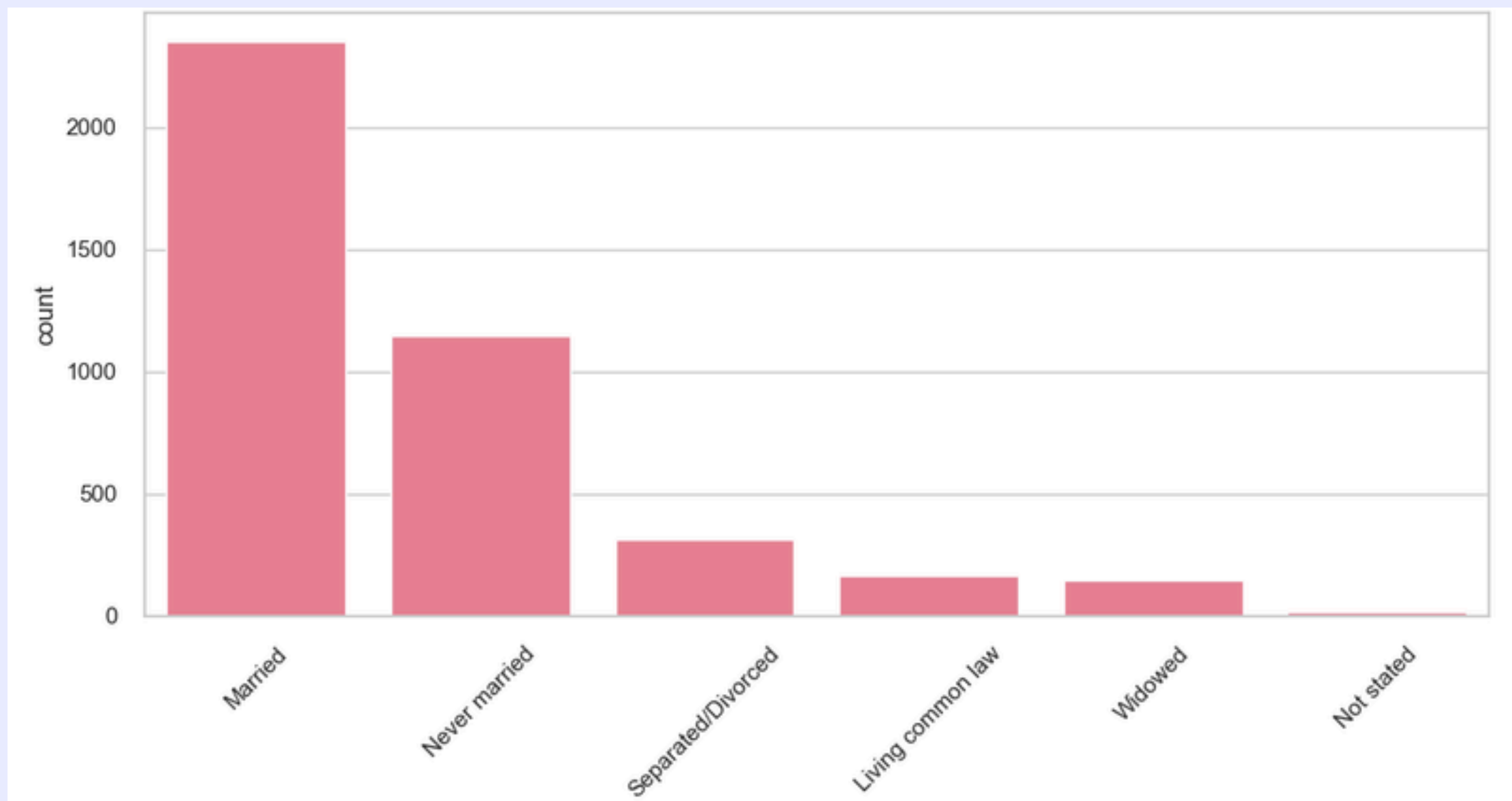
To encourage civic engagement by promoting awareness of community resources and fostering a sense of belonging among immigrants.

BASIC INFORMATION

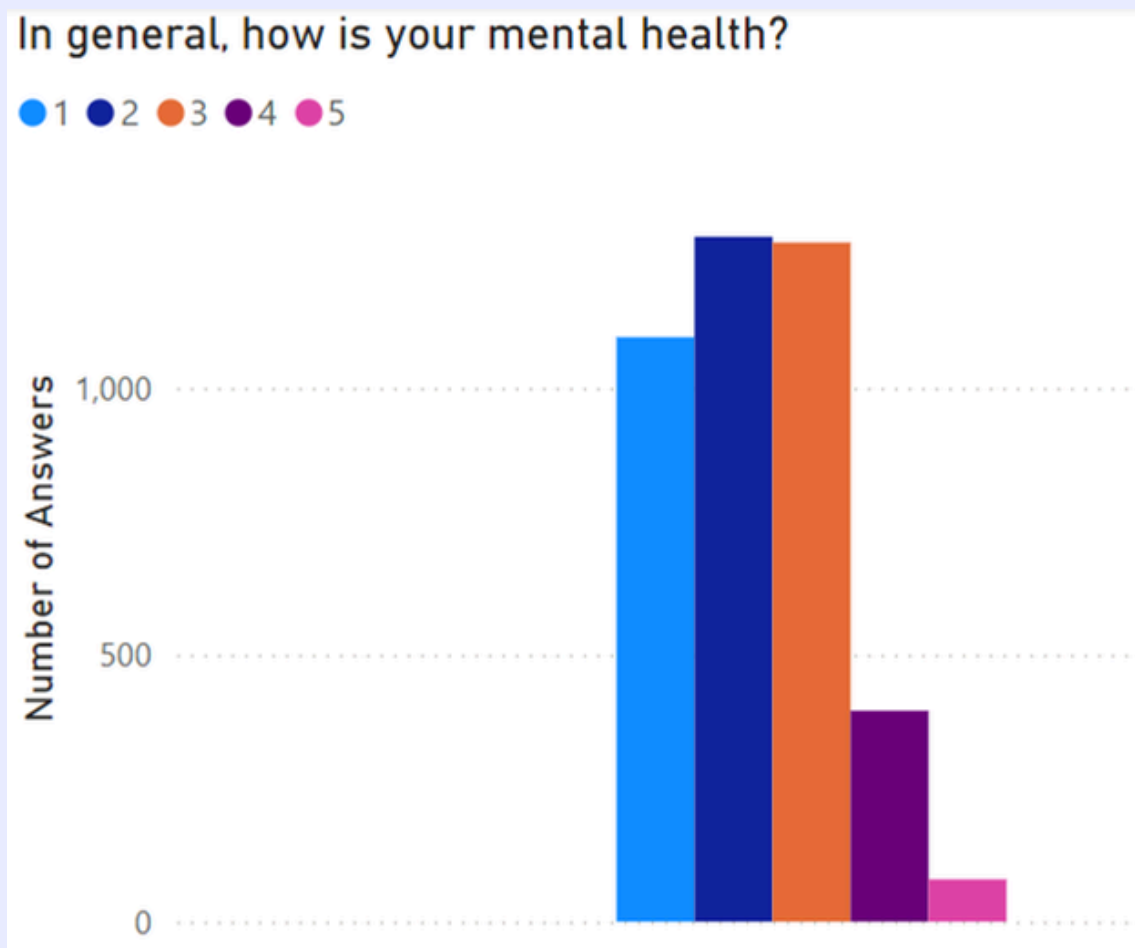
The sample is nearly evenly split by gender, with slightly more men than women. Seniors and middle aged adults make up most of participants, and majority of respondents have a partner. Most participants report their mental health to be relatively good across all age groups. Most people feel their mental health status have not changed much for the past 12 months. Overall, the data suggests that self-perceived health tends to decline with age, though a significant portion of older adults still consider their health to be good or very good.



Graph 1 Age and Gender of immigrants in the survey

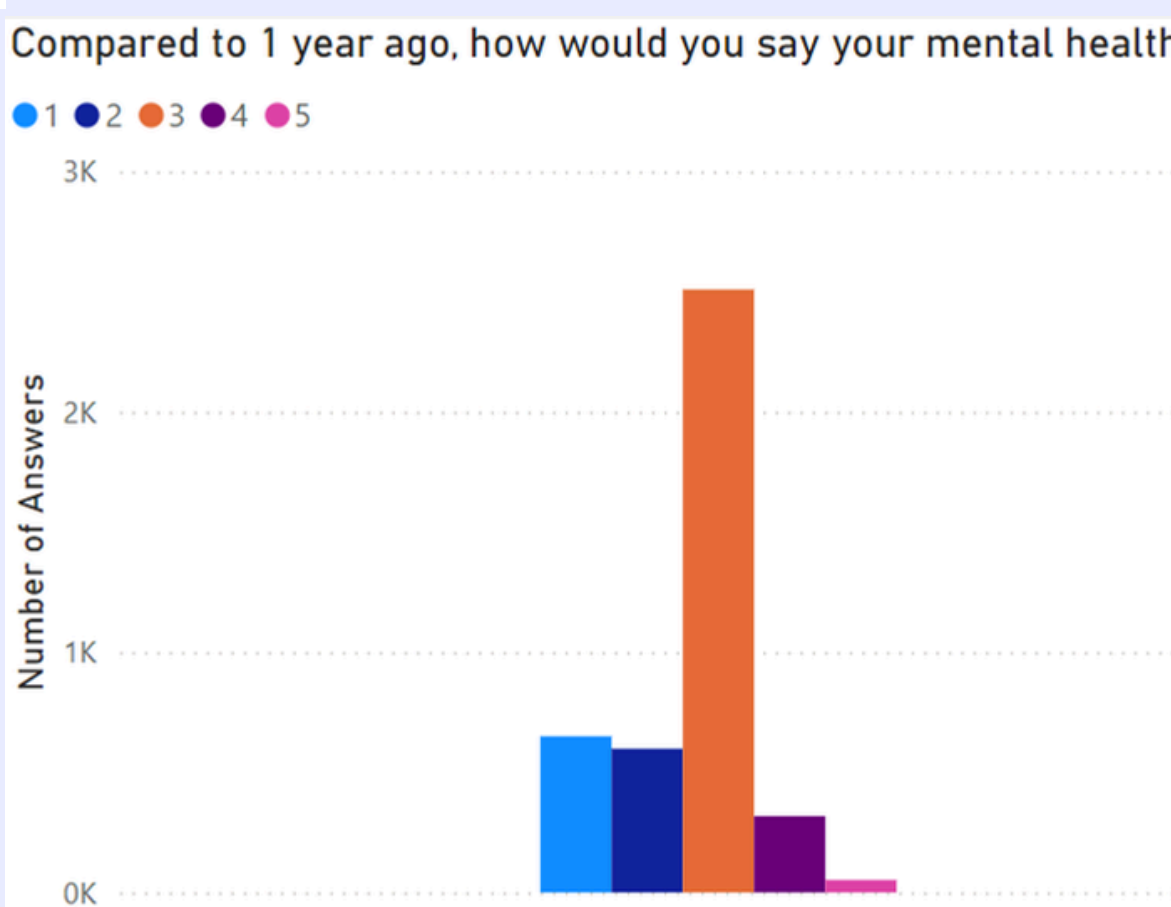


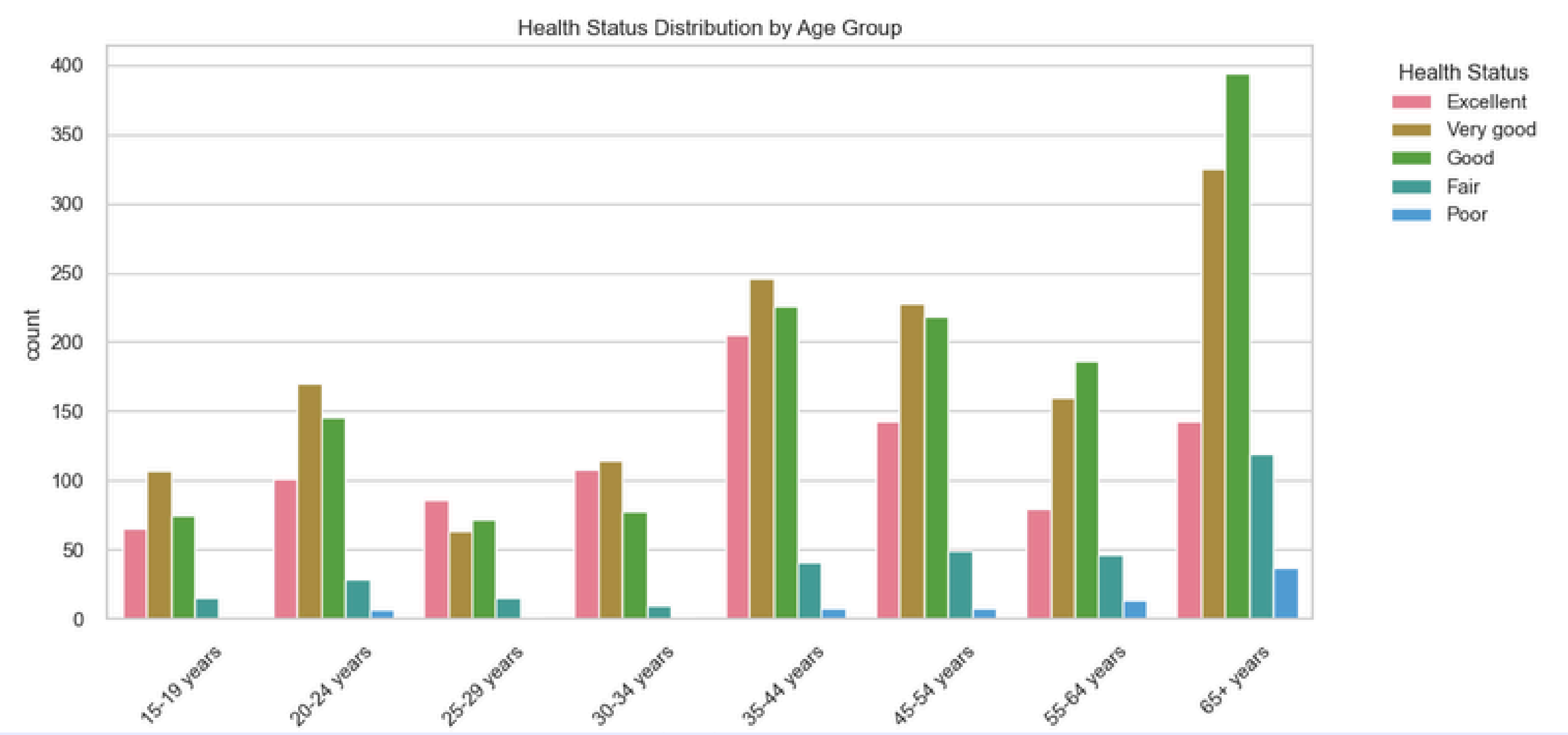
Graph 2 Marital Status of immigrants in the survey



Graph 3 Self-Perceived Mental Health (1 - Excellent, 3 - Good, 5 - Poor)

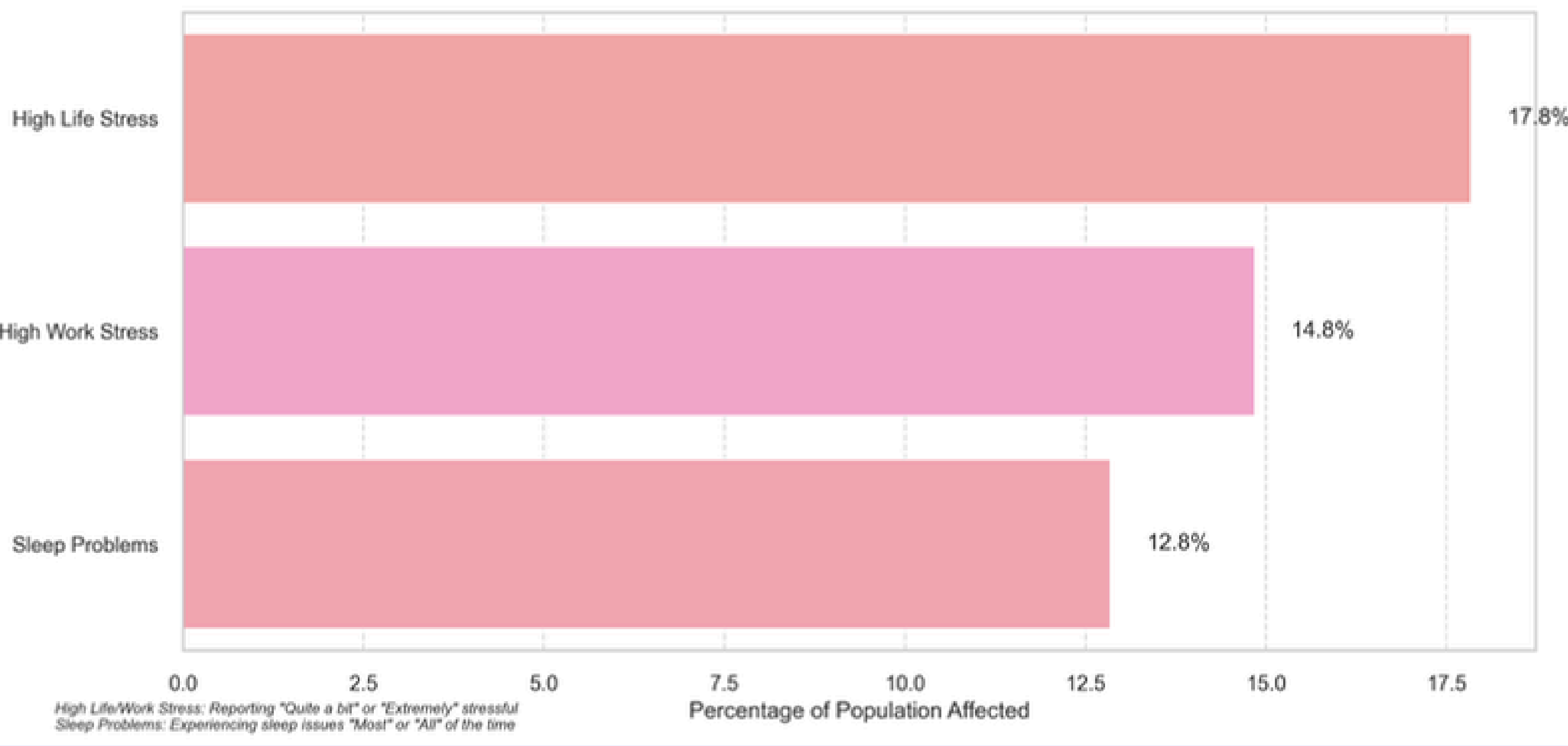
Graph 4 Self-Perceived Mental Health Compared with 1 year ago (1 - Much Better, 3 - Same, 5 - Much Worse)





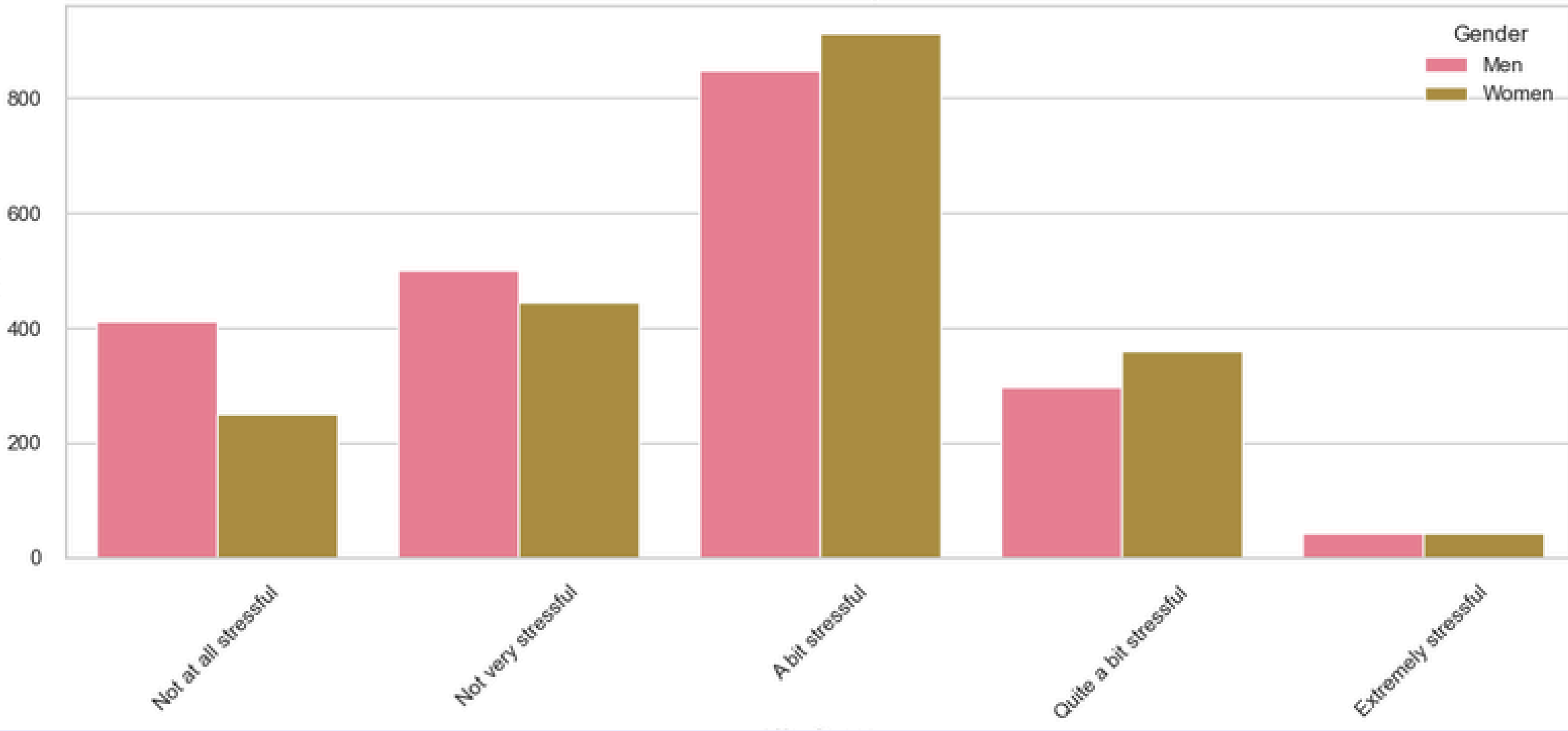
Graph 5 Self-Perceived Mental Health across Age Groups

STRESS AND SLEEP



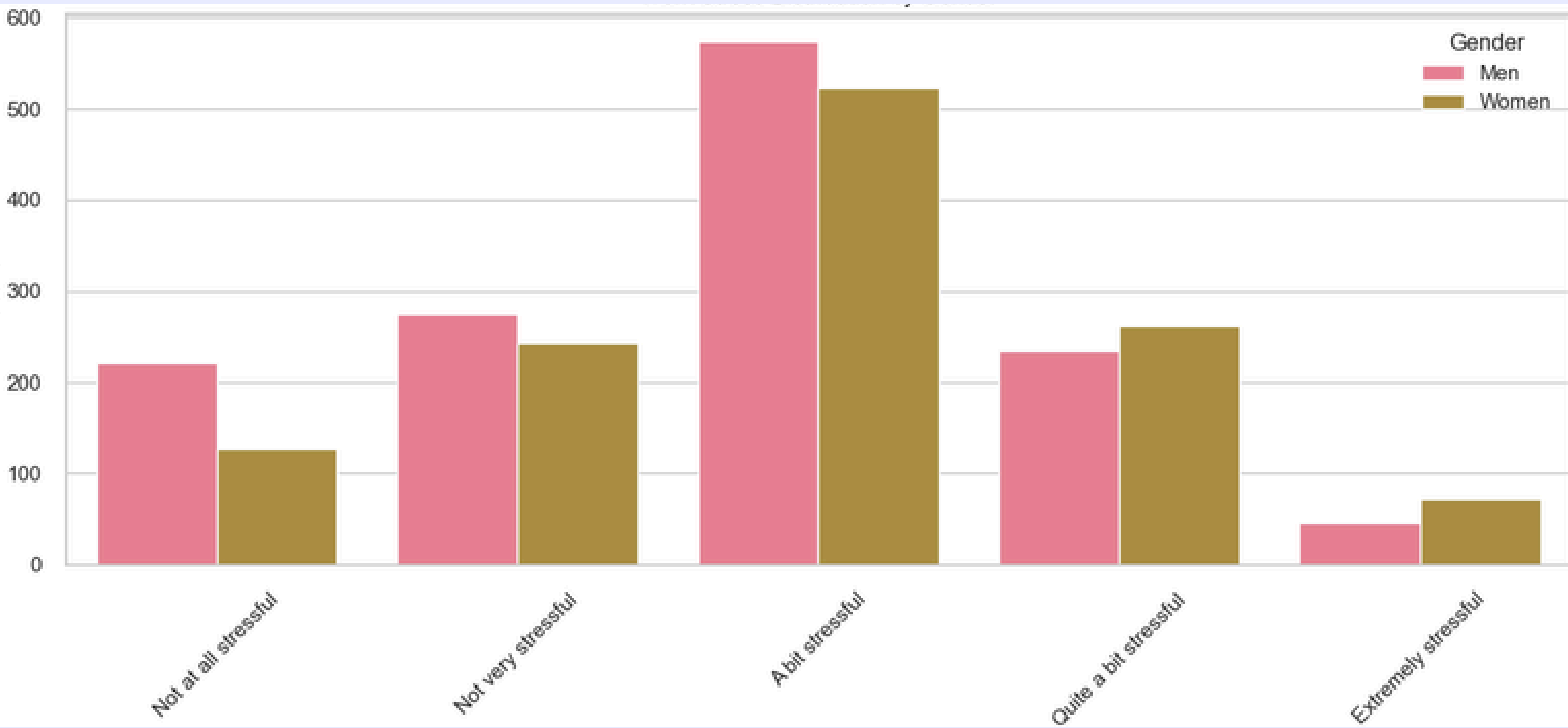
Graph 1 Overview of Common Mental Health Challenges

This chart highlights common mental health challenges in immigrants. About 17.8% of people report experiencing high levels of life stress, making it the most prevalent issue. High work stress affects 14.8% of individuals, while 12.8% report frequent sleep problems. These figures reflect the proportion of the population facing significant day-to-day mental health strains, such as feeling extremely stressed or having ongoing trouble sleeping.

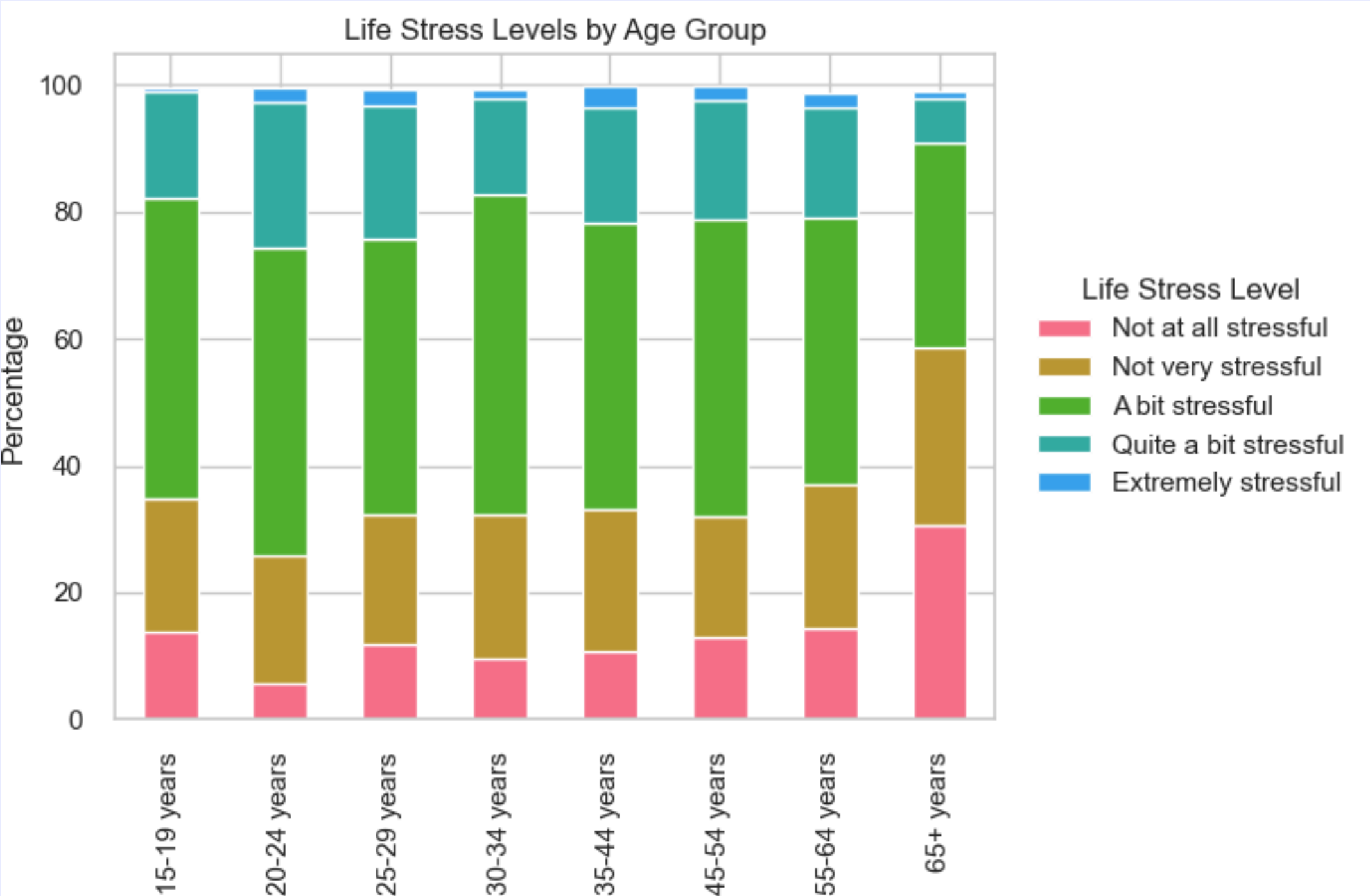


Graph 2 Life Stress by Gender

These charts show how men and women experience life and work stress differently. For life stress, both genders most commonly report being “a bit stressful,” but women are slightly more likely than men to rate their lives as “quite a bit” or “extremely” stressful. Men, on the other hand, are more likely to say their life is “not at all” or “not very” stressful. For work stress, the pattern is similar: most people report being “a bit stressful,” with women again more likely to report higher stress levels than men. Overall, women tend to report higher stress levels than men in both life and work settings.

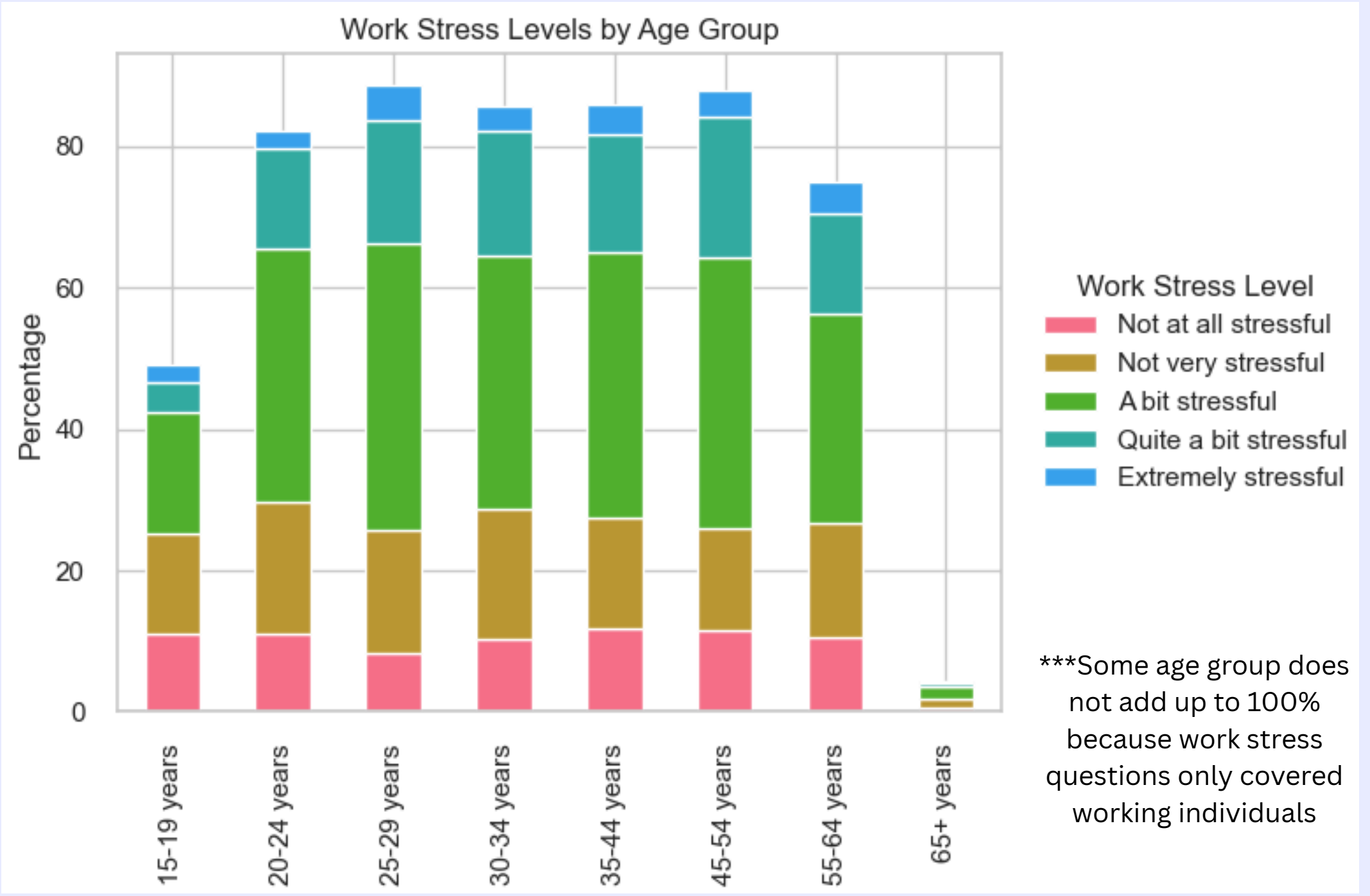


Graph 3 Life Stress by Gender



Graph 4 Life Stress by Age

For life stress, most people across say their lives are “a bit stressful.” However, younger and middle-aged adults are more likely to report higher stress levels, while older people report significantly lower stress. For work stress, younger and working-age adults (20–54) are more likely to report “quite a bit” or “extremely” stressful work lives. Stress levels drop off sharply for the 65+ group, likely reflecting retirement. Overall, work and life stress appears highest in early and mid-adulthood, decreasing notably in later years.



***Some age group does not add up to 100% because work stress questions only covered working individuals

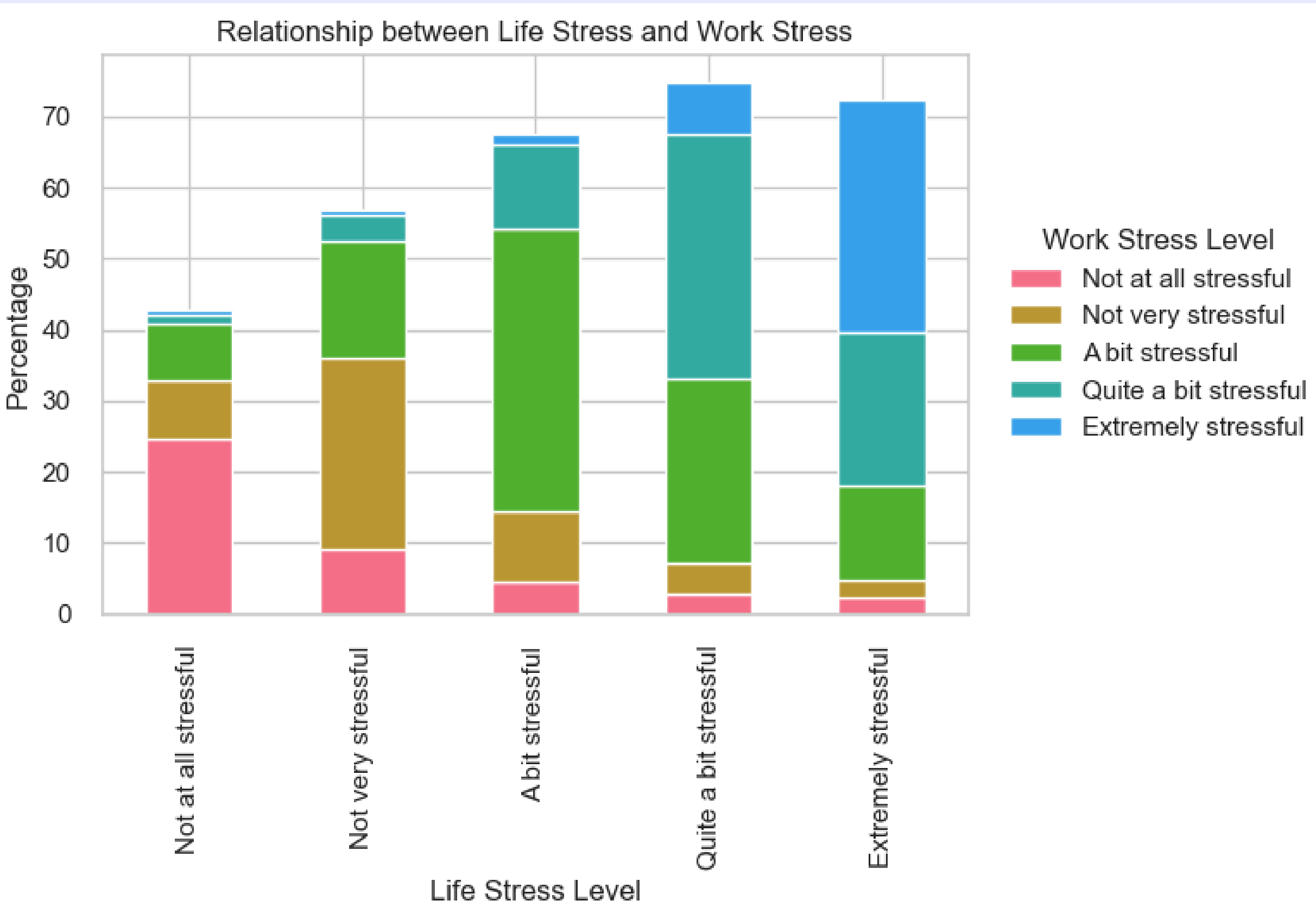
Graph 5 Work Stress by Age

These charts highlight the strong connection between life stress, work stress, and mental health. As individuals report higher levels of life stress, their levels of work stress rise accordingly.

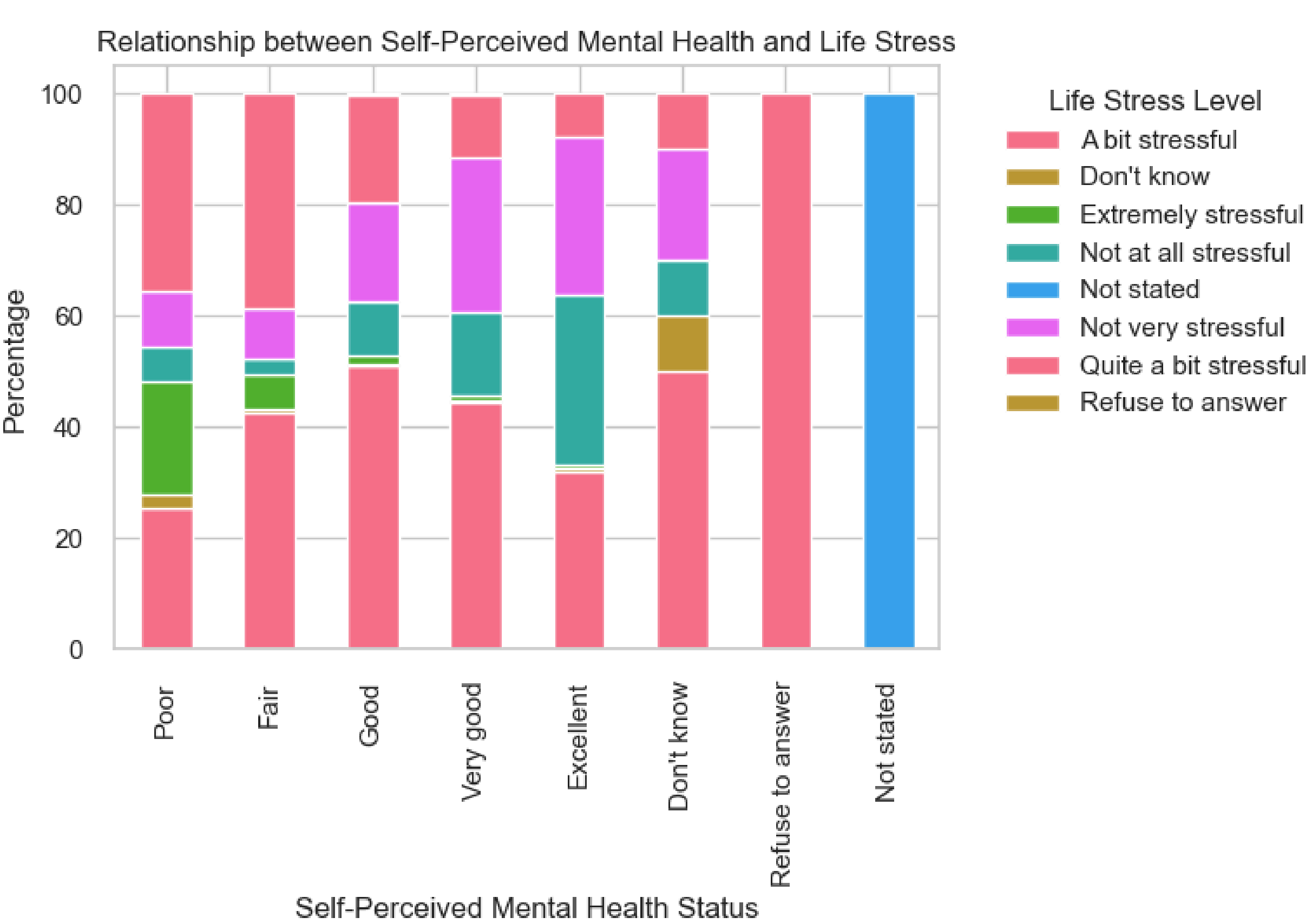
For example, among those who say life is “extremely stressful,” the majority also rate their work as “extremely” or “quite a bit” stressful. On the flip side, those with low life stress typically experience minimal work stress, suggesting the two are closely linked.

The second chart dives into how life stress aligns with self-perceived mental health. People who report “poor” or “fair” mental health are far more likely to experience higher levels of life stress. In contrast, those who describe their mental health as “very good” or “excellent” are more likely to report lower stress levels—or even no stress at all.

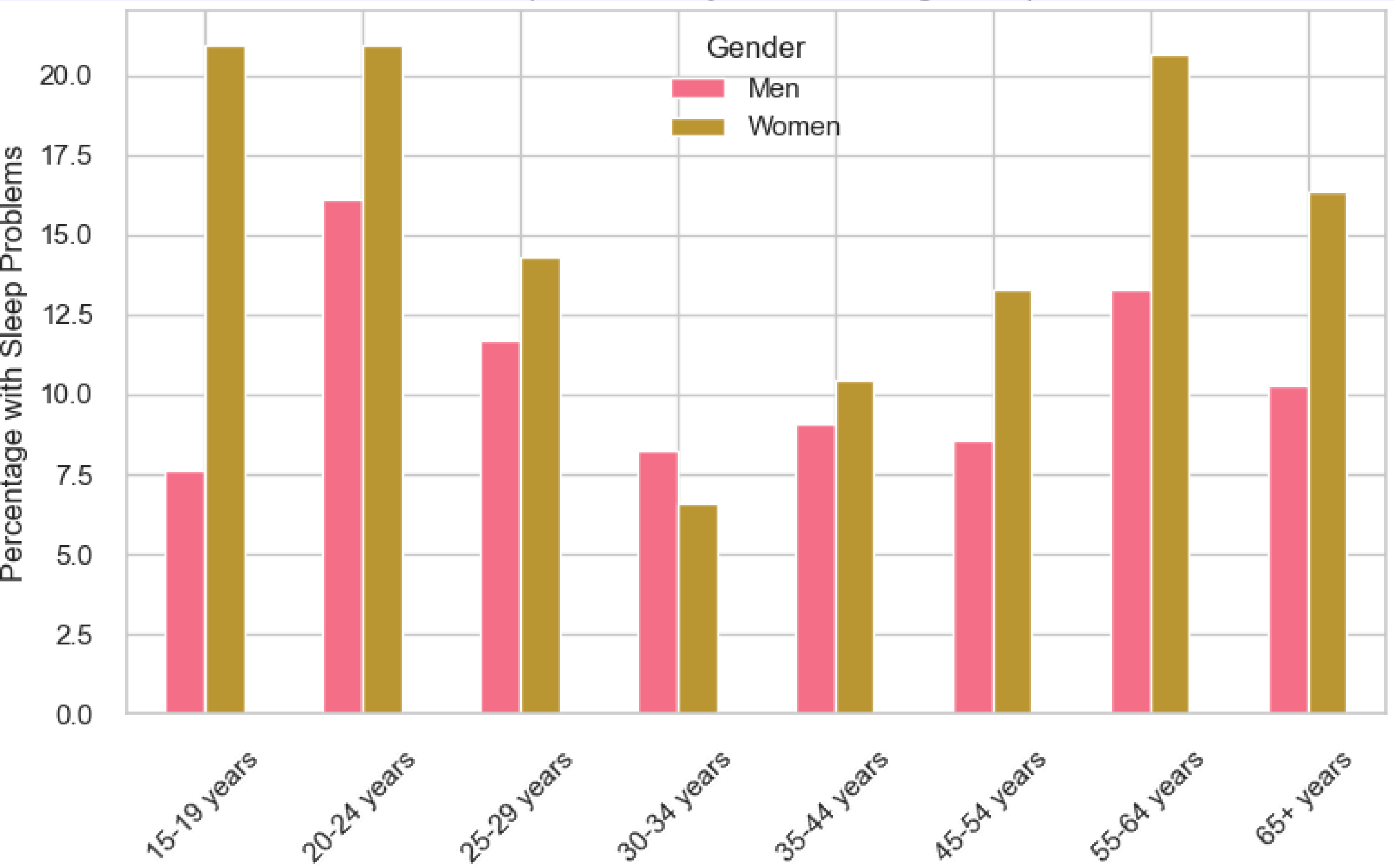
These patterns underscore the impact of daily stress on overall mental well-being and highlight the importance of stress management in promoting better mental health outcomes.



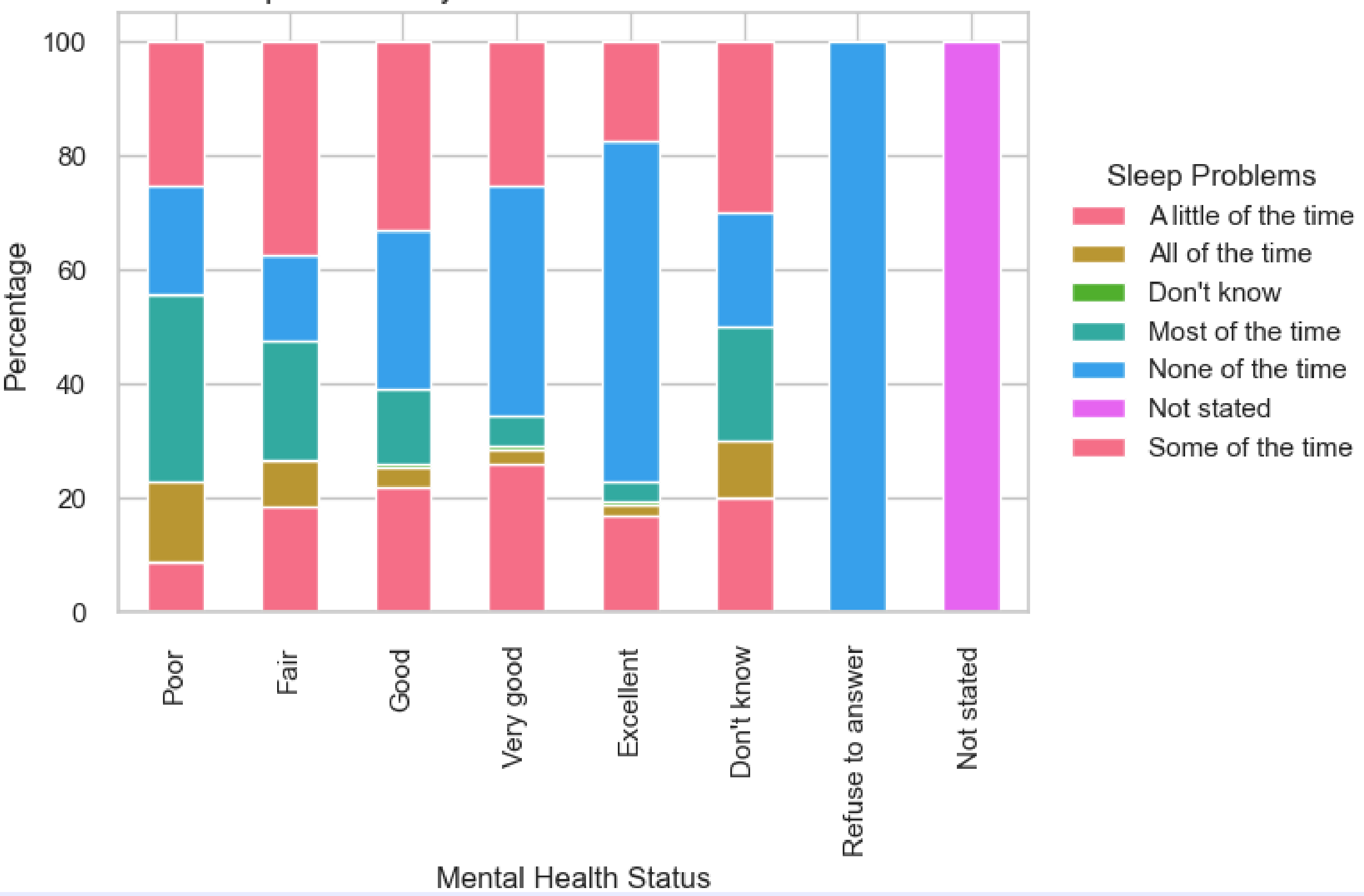
Graph 5 Relationship between Life Stress and Work Stress



Graph 6 Relationship between Self-Perceived Menta Health and Life Stress



Graph 6 Sleep Problems by Age and Gender



Graph 7 Sleep Problems and Mental Health

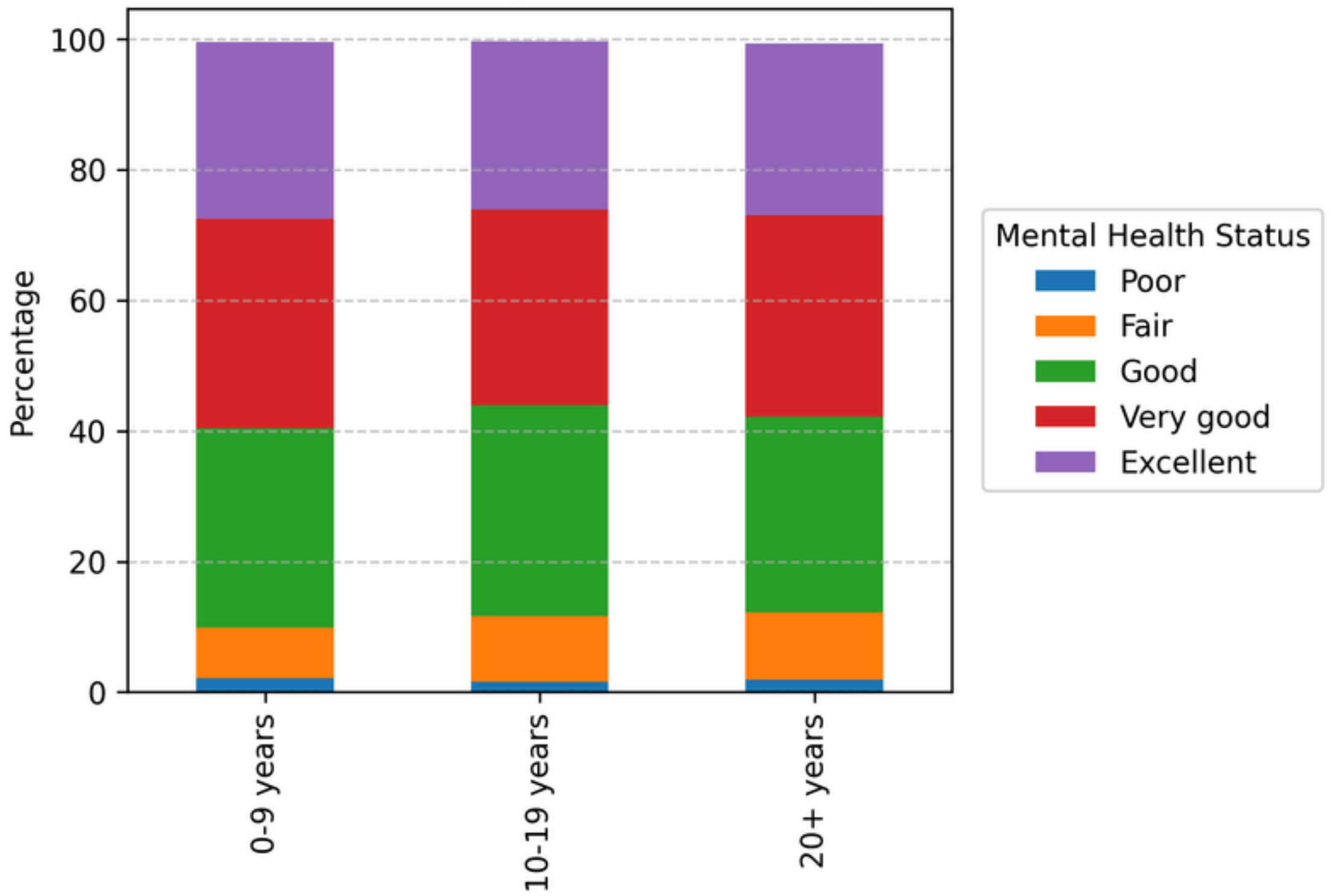
These charts explore how sleep problems vary across different groups and how they relate to mental health.

In the first chart, we see that women consistently report more sleep problems than men, especially for female teenagers and young adults (15–24) and older ladies (55–64), with over 20% in these groups experiencing frequent sleep issues. For men, it seems that young males (20-24) are the most likely to have sleep problems. Both middle aged male and females showed improvement in sleep quality.

The second chart shows a strong link between sleep quality and mental health. Individuals with “poor” or “fair” self-rated mental health are far more likely to experience sleep problems “most” or “all of the time.” On the other hand, those with “excellent” mental health mostly report no sleep problems or issues only “a little of the time.” This suggests that better mental health is closely tied to better sleep.

Altogether, these visuals highlight the important connection between sleep and mental health. Addressing stress and promoting sleep hygiene may be key to improving sleep health in the immigrants.

SELF-REPORTED MENTAL HEALTH SYMPTOMS SUMMARY



Graph 1 Mental Health by Years since Immigration

Num of Depression		Num of Anxiety	
SCRDDEP	Count of SCRDEP	SCRDGAD	Count of SCRDEGAD
Yes	2072	Yes	1869
No	1961	No	2096
N/A	109	N/A	177
Total	4142	Total	4142

Num of Mania		Num of Social Phobia	
SCRDMIA	Count of SCRDMIA	SCRDSOP	Count of SCRDSOP
Yes	914	Yes	839
No	3128	No	3211
N/A	100	N/A	92
Total	4142	Total	4142

Table 1 Overview of Self-Reported Mental Health Symptoms in Surveyed Immigrants

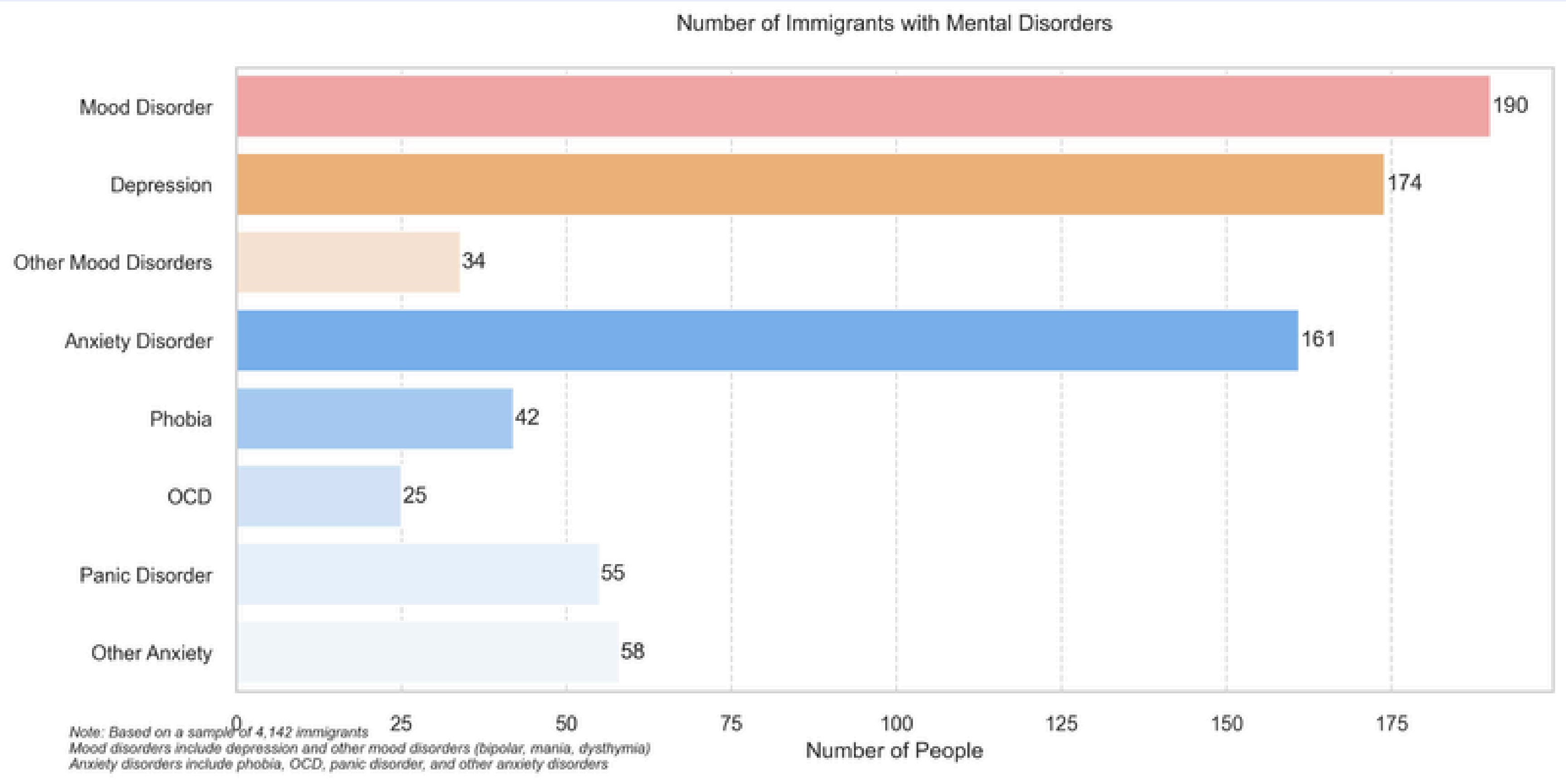
Overall, the majority of immigrants across all timeframes rate their mental health positively. Meanwhile, the percentages of negative mental health ratings (Fair and Poor) remain relatively consistent across all groups. These results suggest that most immigrants report similar levels of positive and negative mental health regardless of how long they have been in the country.

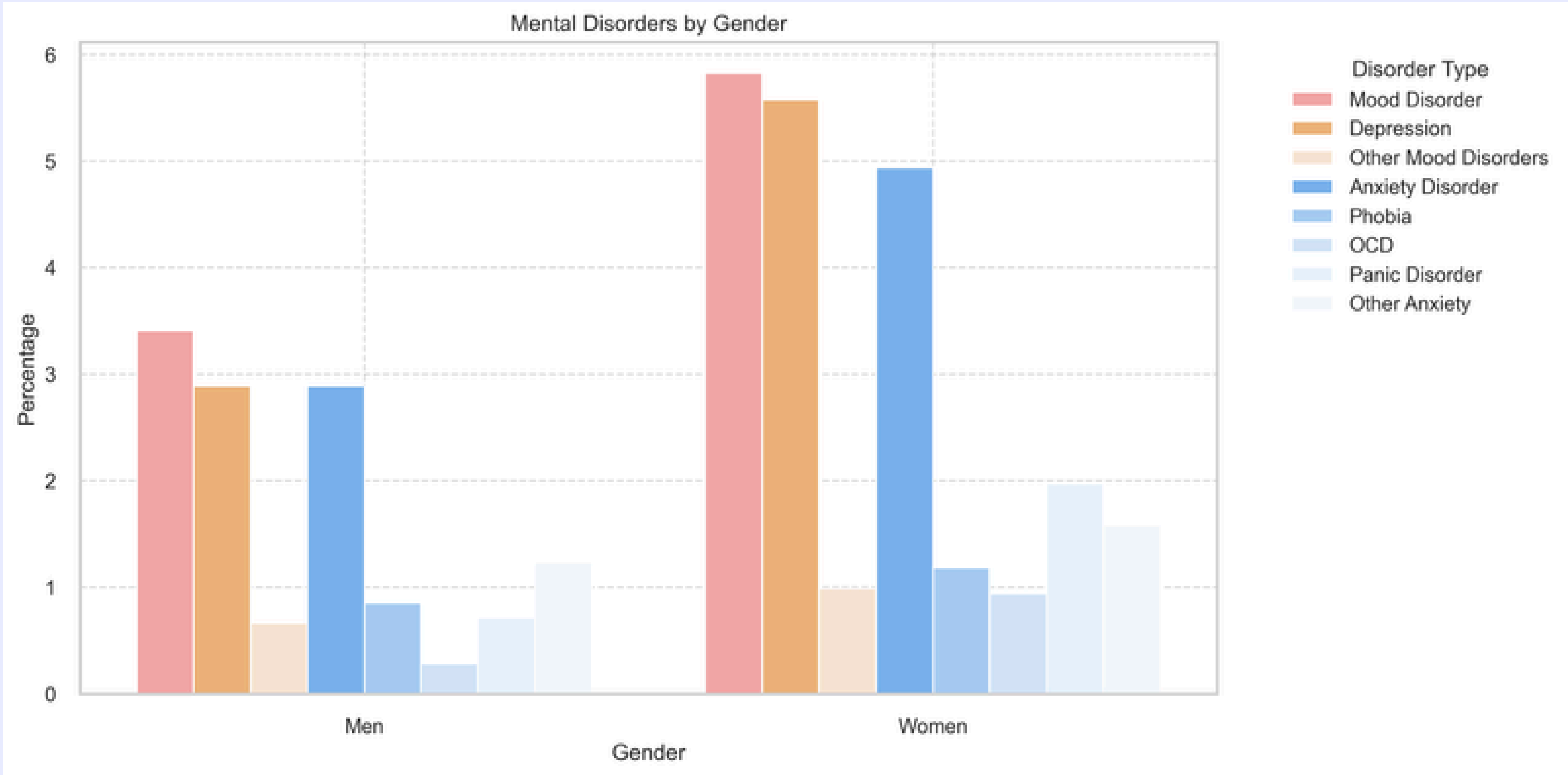
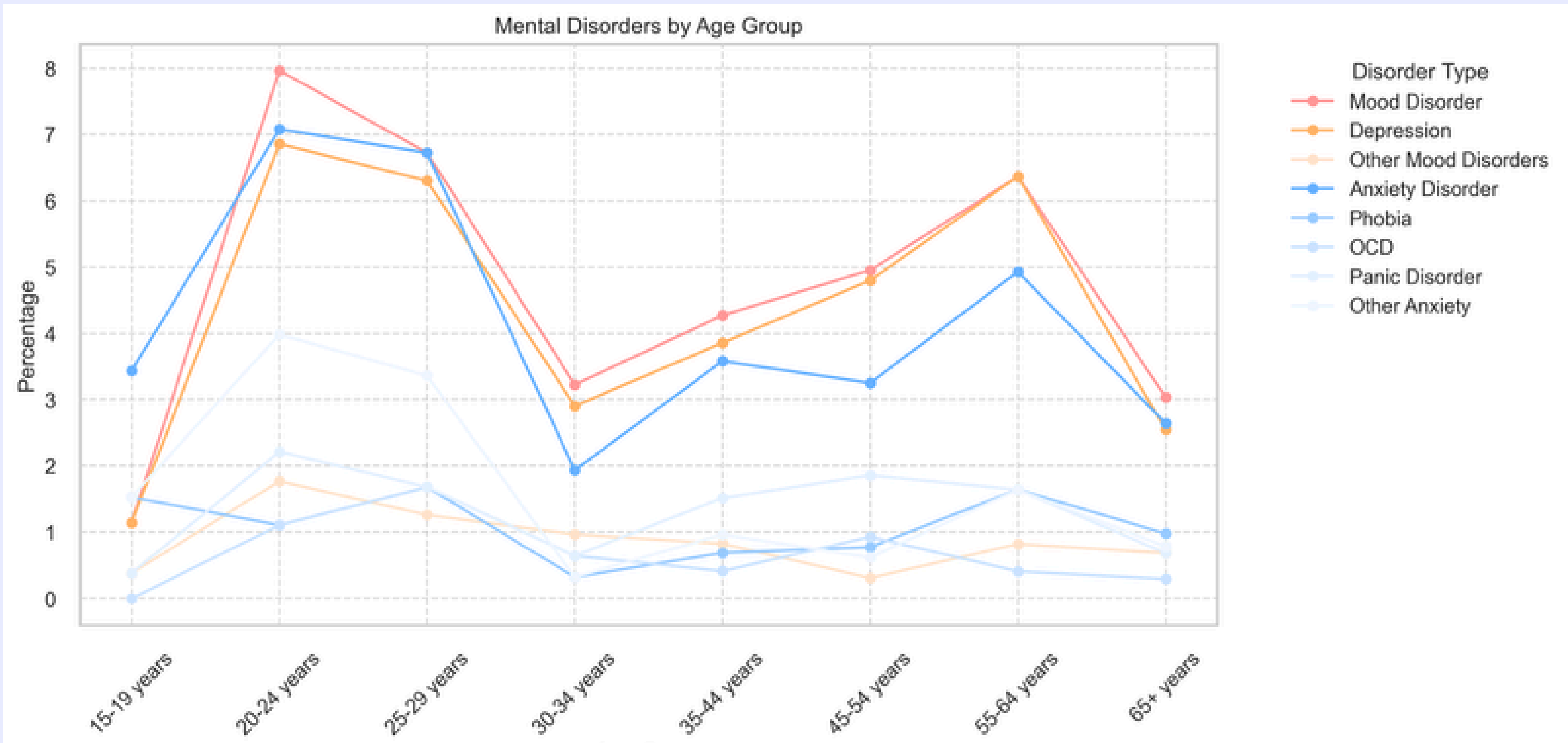
According to a self-report screener, out of 4,142 participants, about half self-reported having experienced depression symptoms (2,072), and a slightly smaller number (1,869) reported anxiety symptoms. Fewer participants reported experiencing mania (914) or social phobia (839). Although immigrants rated their mental health positively overall, episodes of mental issues are still quite prevalent in the immigrant communities. Overall, depression and anxiety symptoms appear to be the most commonly reported mental health concerns.

DIAGNOSED MENTAL DISORDER SUMMARY

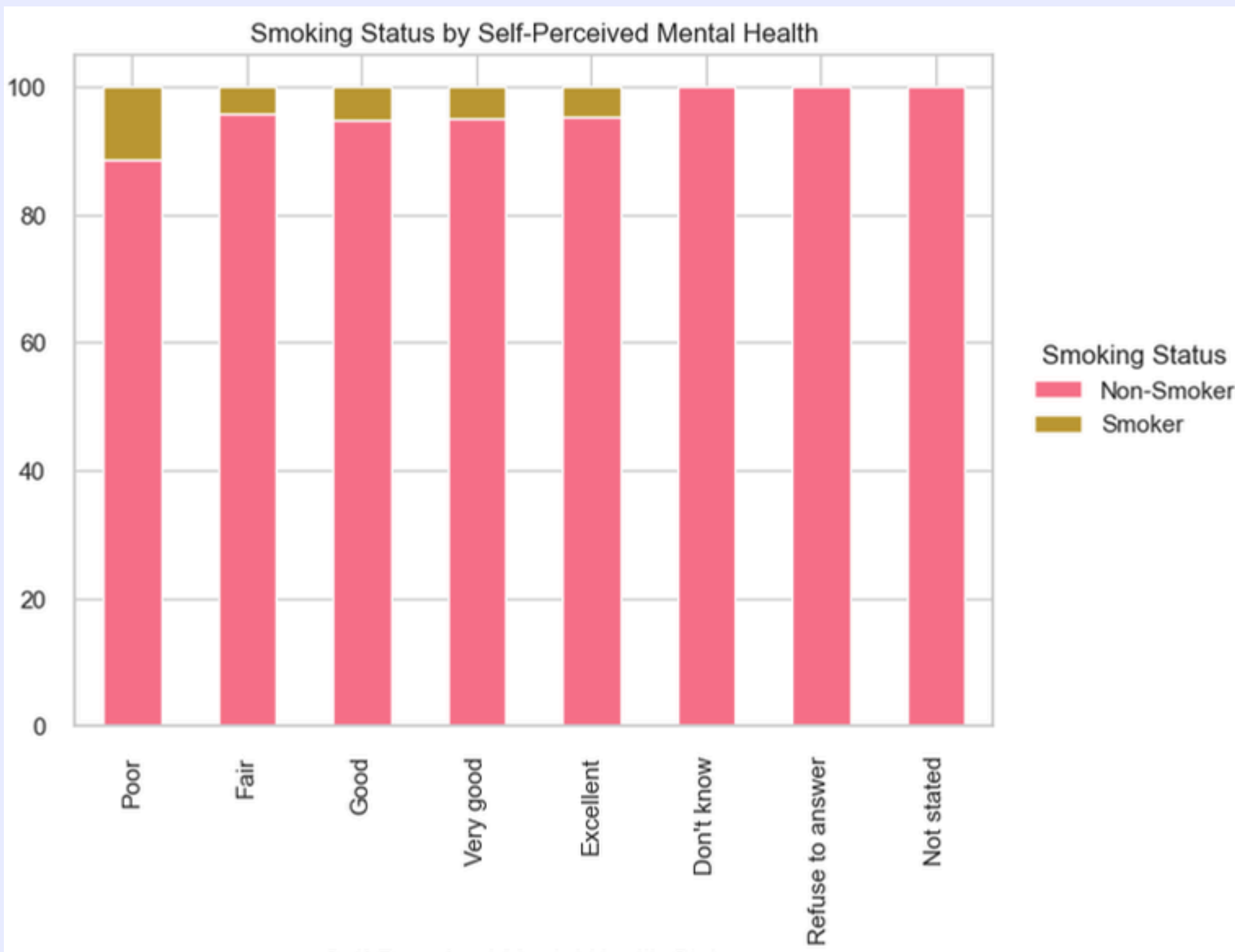
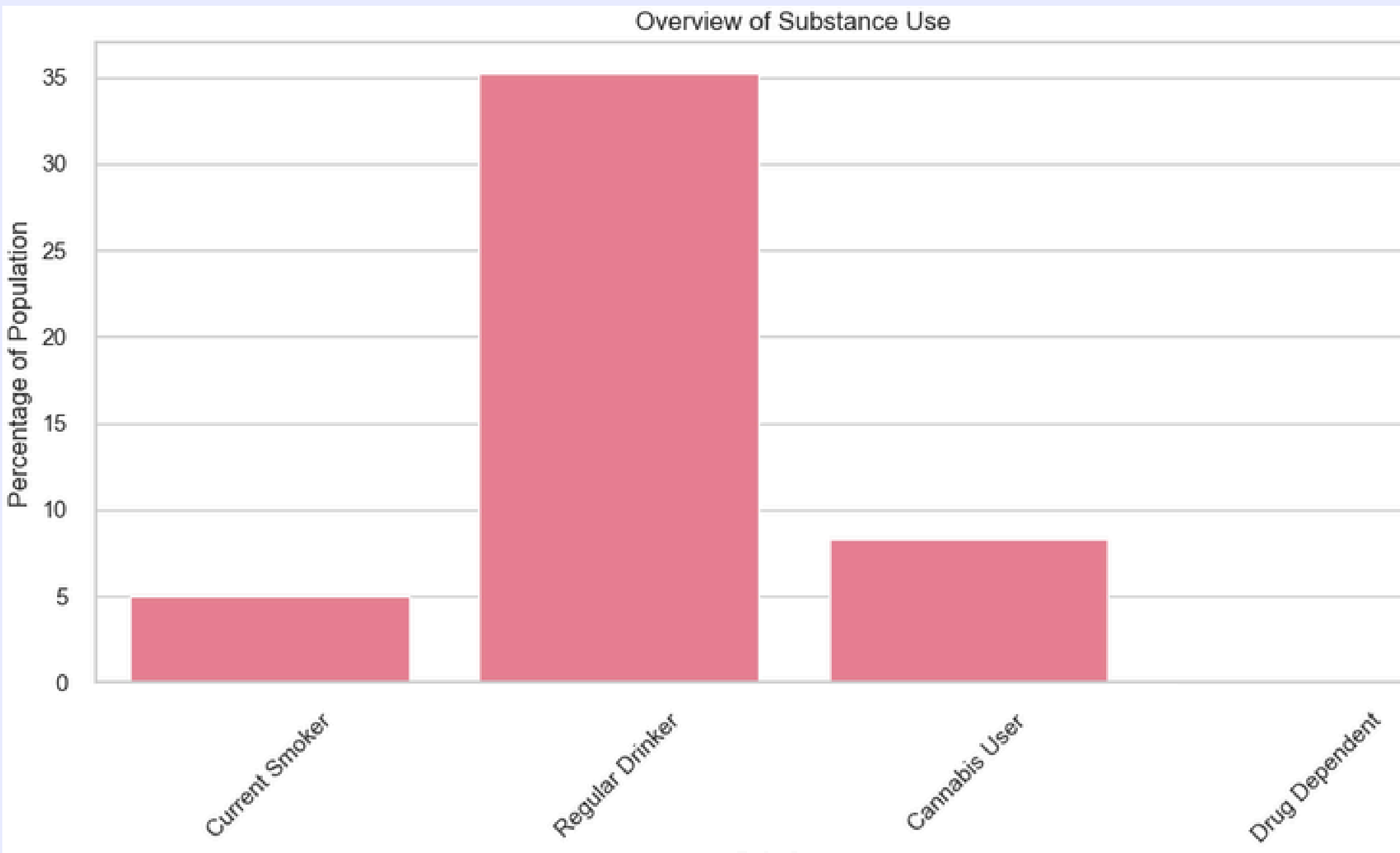
Mood disorders, particularly depression, are the most commonly reported mental health conditions, followed by anxiety disorders. The highest rates are observed among young adults aged 20–24. Rates tend to decline in older age groups, though there is a noticeable increase again around the 55–64 age range. This suggests that young adulthood and pre-retirement years may be particularly vulnerable periods for mental health challenges. Women consistently report higher rates of nearly every disorder. For example, around 6% of women report mood disorders, compared to 3.4% of men. Women also show significantly higher rates of phobia and panic disorder. Together, these charts suggest that young immigrant women are especially vulnerable to mental health challenges.

The actually diagnosed cases of mental disorders are relatively low, even though quite a significant number of immigrants reported experiencing symptoms. For example, about 50% of immigrants reported symptoms of depression, but only about 5% of them have a diagnosis. This is unlikely to be a case of massive under-diagnosis, because the majority of immigrants perceived their mental health condition as at least good. The more likely answer is that immigrants are facing quite some stress, but not bad enough to cause a formal diagnosis. This can be confirmed by looking at stress statistics we presented earlier. Over 50% of immigrants reported facing some level of life and work stress, and about 15% of them reported high levels of stress. As a result, mental health support for immigrants should focus on lessening general life and work stress, such as nutrition, sleep hygiene, social engagement, healthy life style.



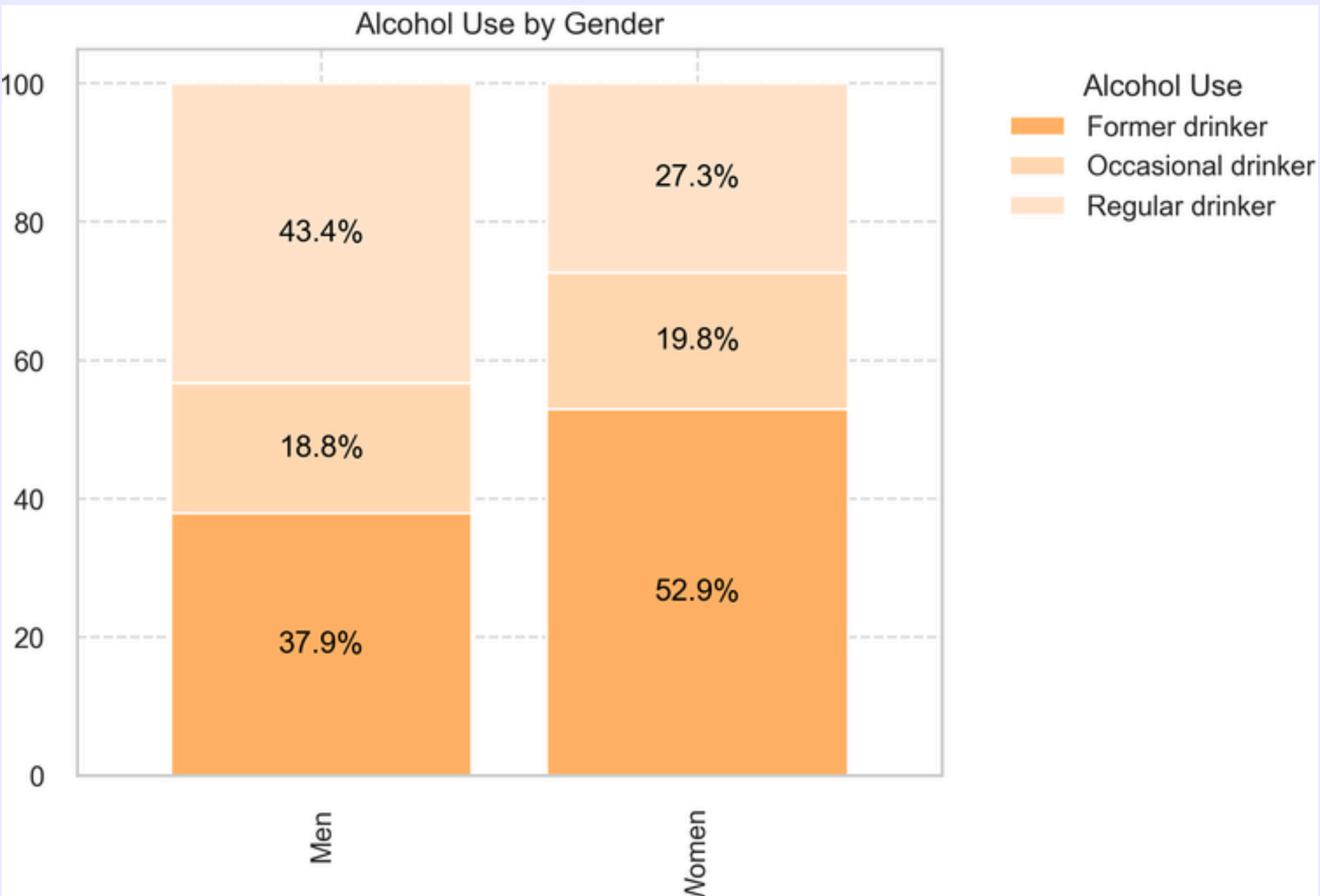
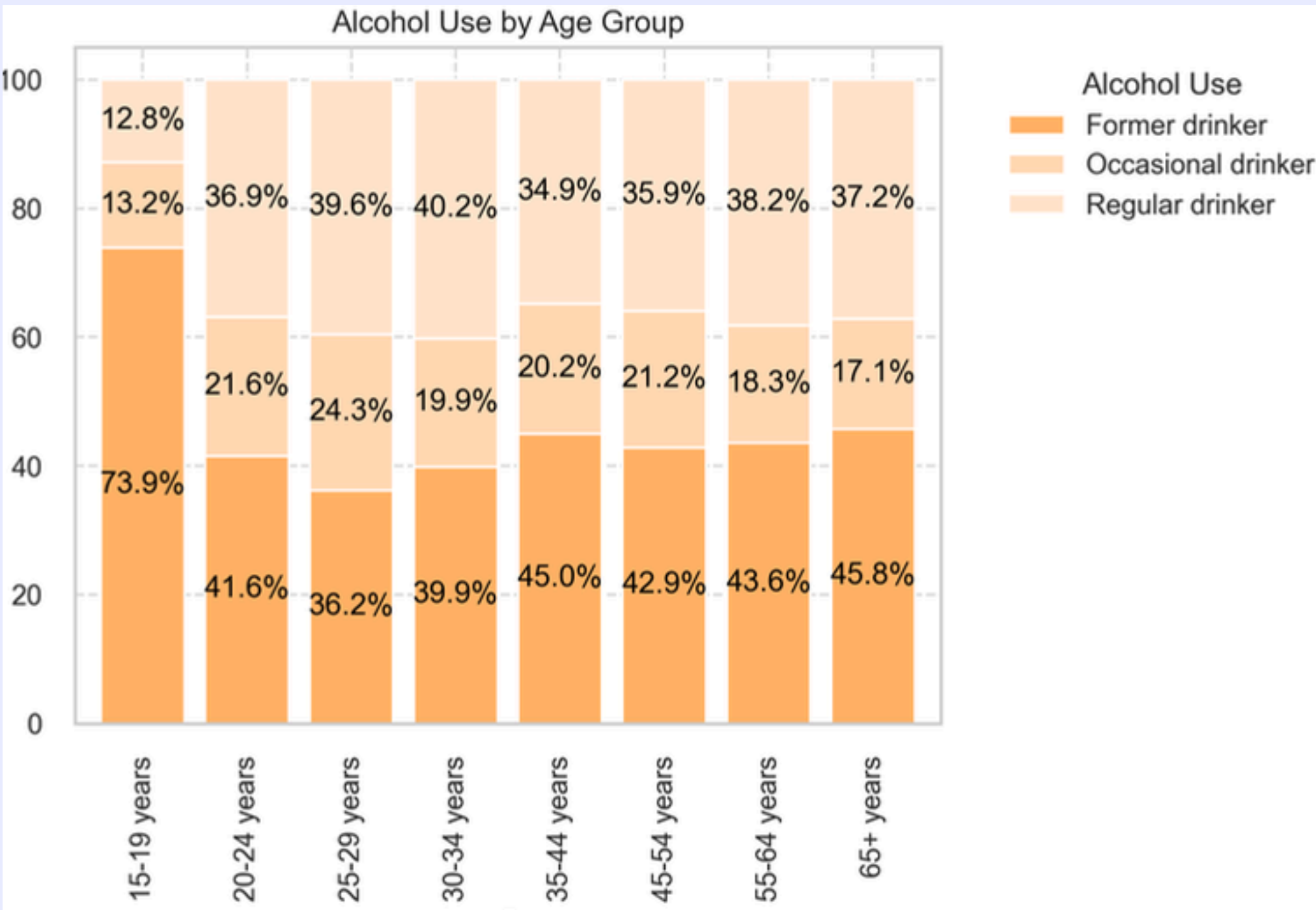
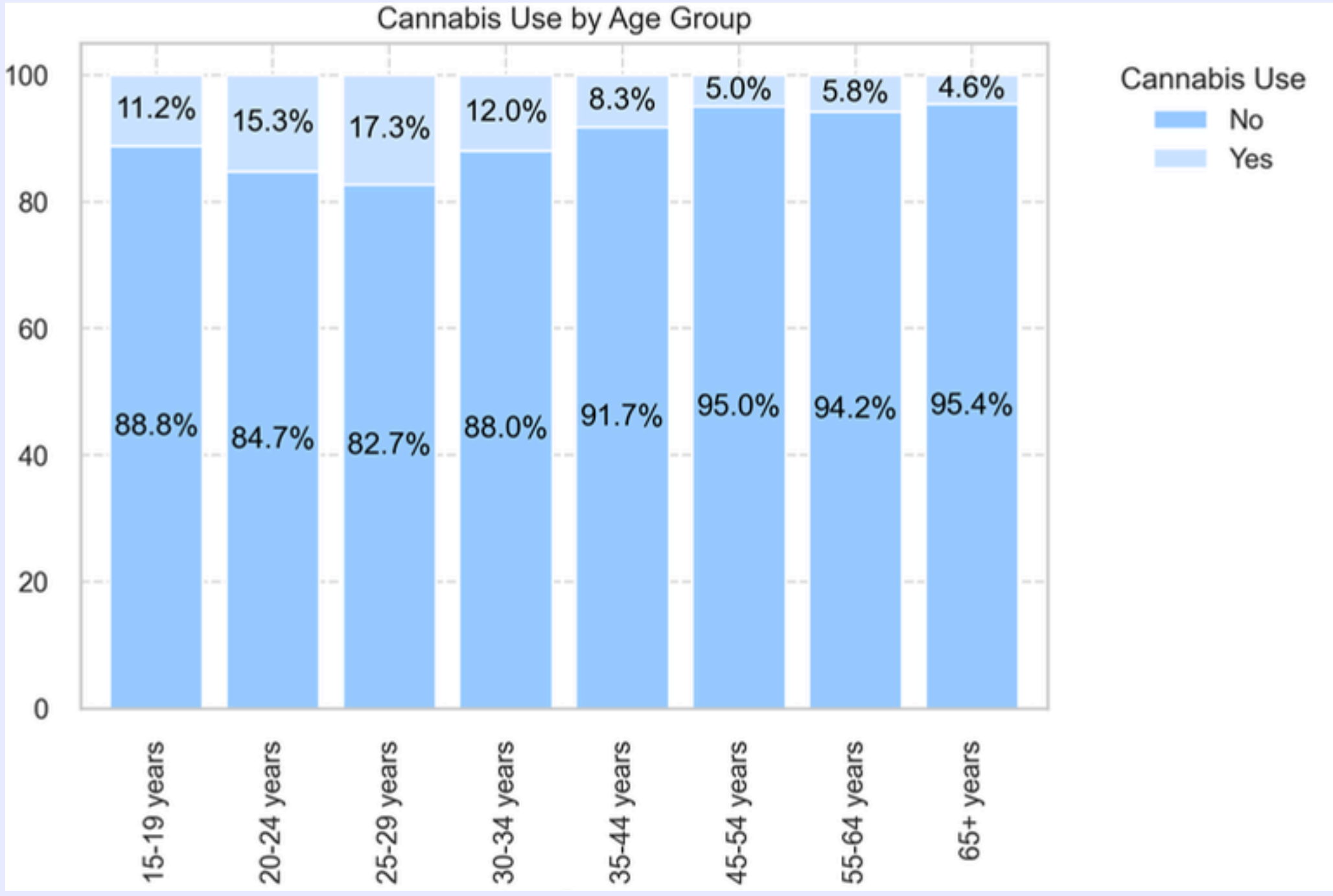
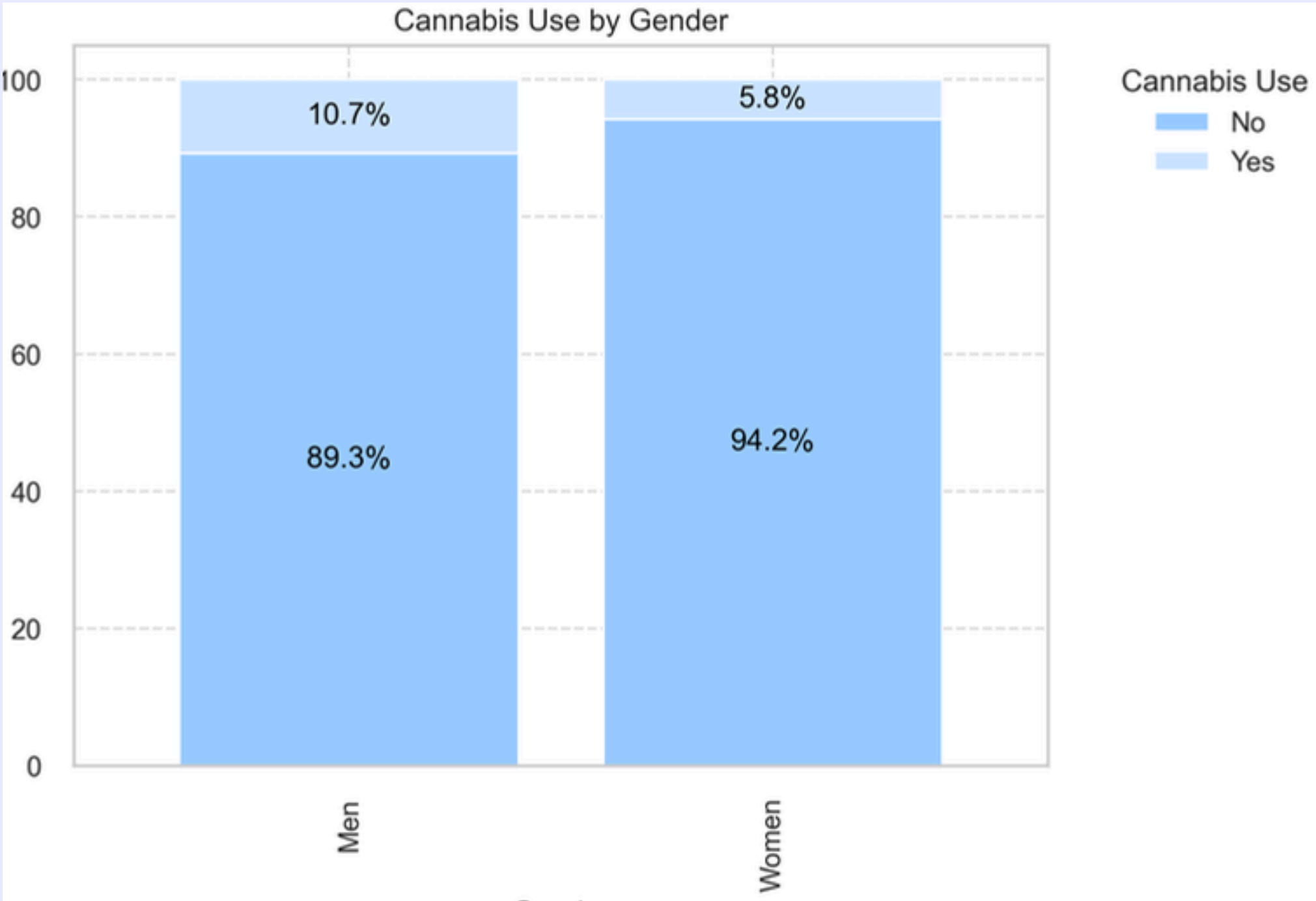
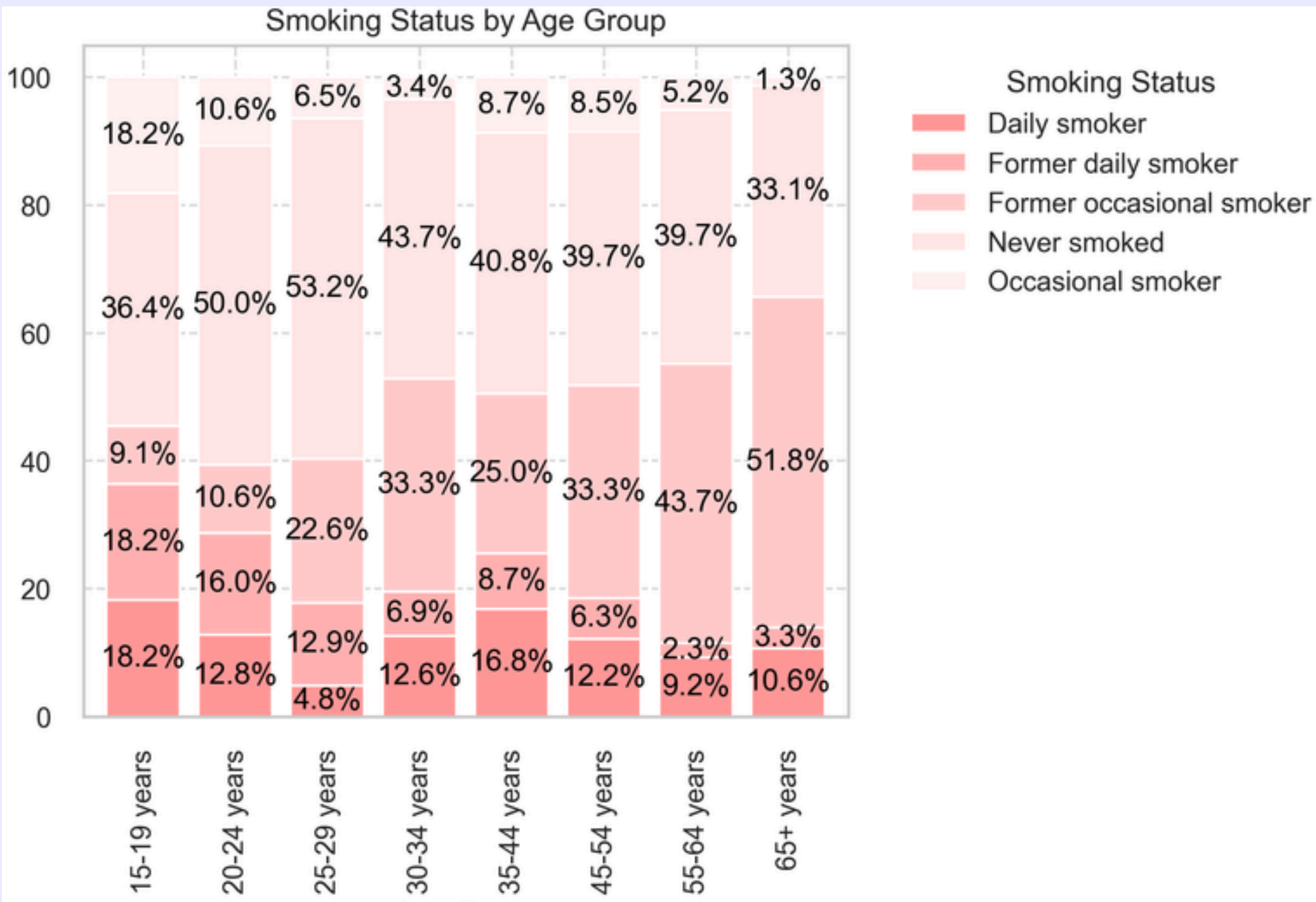
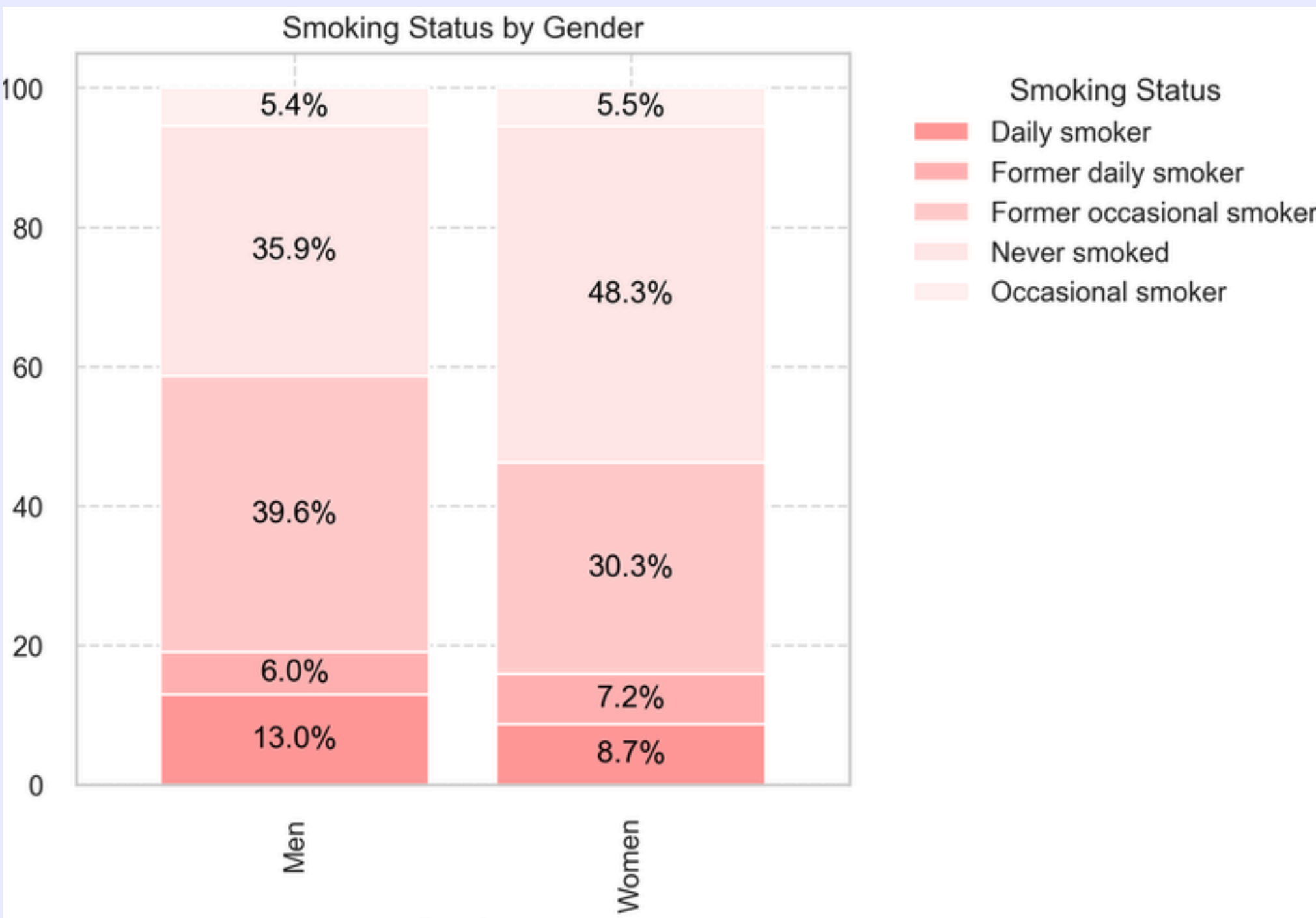


SUBSTANCE USE



Substance use can be an important indicator for mental health because poor mental health is clearly associated with more use such as smoking. One possibility is that people with poor mental health may use smoking as self-medication or a coping method.

Substance use patterns in Canada show that regular alcohol use is the most common followed by cannabis use and smoking, while actual drug dependence being almost non-existent. Alcohol use peaks in young adulthood (25–34) with around 40% regular drinkers, while youth (15–19) show the highest rate of former drinkers (74%), suggesting early quitting. In this case, it is important to intervene early to prevent early drinking. Men are more likely to be regular drinkers (43%) and cannabis users (11%) than women (27% and 6%, respectively), while women are more likely to be former drinkers and have never smoked. Cannabis use is highest among those aged 20–29 (15–17%) and drops sharply with age, falling below 6% after age 45. Smoking is more prevalent among men, with 19% being current or former daily smokers, compared to 16% of women. Smoking tends to rise for youth and middle aged men, probably suggesting that youth experiments with smoking for coolness or defiance, while middle aged men use smoking as coping for stress. Overall, substance use trends reveal age and gender-based differences, with younger groups experimenting more and older groups showing sustained or former use.



SUICIDE IDEATION

Suicide is an important issue, and about 5% immigrants have thought about it in their life. Suicide ideation rates are highest among young adults aged 20–29 and lowest among seniors aged 65+. Age trends show suicide ideation generally declines as people get older, though there is a slight uptick in the 55–64 group. Mental health status is a strong predictor. Individuals who rated their mental health as "poor" had the highest rate of suicide ideation, while those who described their mental health as "excellent" had the lowest. Notably, those unsure of their mental health status also reported high rates, emphasizing the importance of awareness and self-assessment in mental health care.

